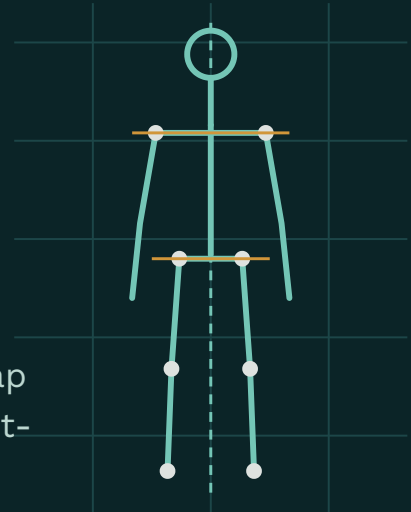




Dheepika Lab

User guide

How to use Dheepika Lab, step by step, for patients and physiotherapists. Includes a map of the whole app, the care loop, a camera set-up guide and answers to common problems.



physiovision-ai-eta.vercel.app

For patients

Tell your physiotherapist where it hurts, do the movement tests with your phone camera, follow your exercise program and track your progress.

For physiotherapists

See every patient of the clinic, review camera estimates, sign the clinical impression and report, and build exercise programs.

Pilot software, not yet approved for patient care. Camera readings are estimates. They are not a diagnosis. Use test accounts only until clinical approval, patient consent, a data-processing agreement and real-device validation are in place. Screens in this guide show the built-in **simulated demo data**. Guide version 3.1 · 7 October 2026.

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What Dheepika Lab is

It does

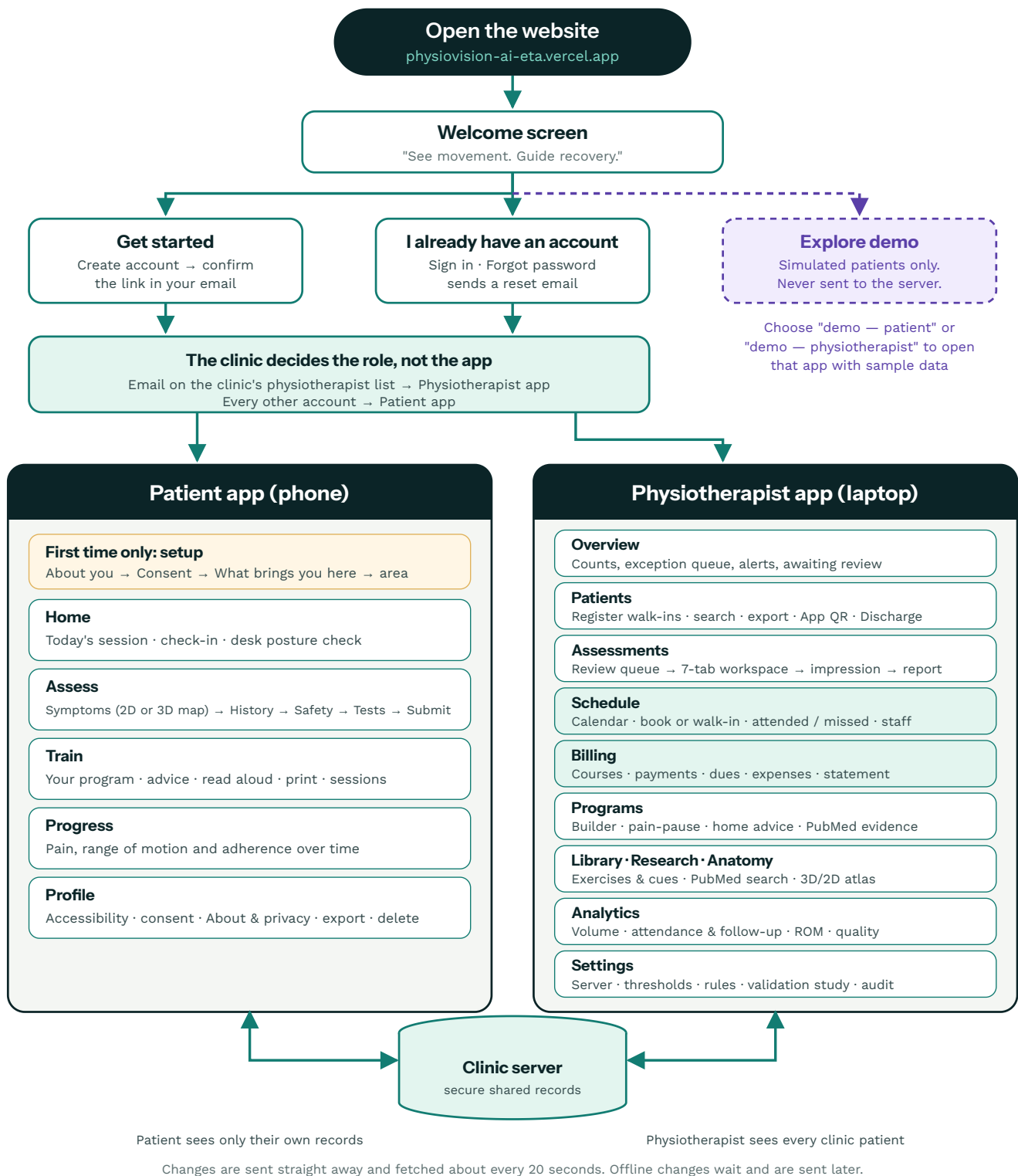
- Record where it hurts and the patient's own answers, exactly as given.
- Ask safety questions and stop routine tests if a warning sign is reported.
- Use the phone or laptop camera to estimate 33 body points, joint angles and posture lines. This runs **on the device**.
- Count exercise repetitions and show the target range.
- Give the physiotherapist a structured review, a report and a program builder.

It does not

- Diagnose any condition or name the injured tissue.
- See bones, joints or vertebrae inside the body. The camera sees only the body surface; there is no X-ray view.
- Measure centimetres from a single camera.
- Store video. Frames are processed in memory and discarded.
- Replace the physiotherapist. Every estimate waits for clinician review.

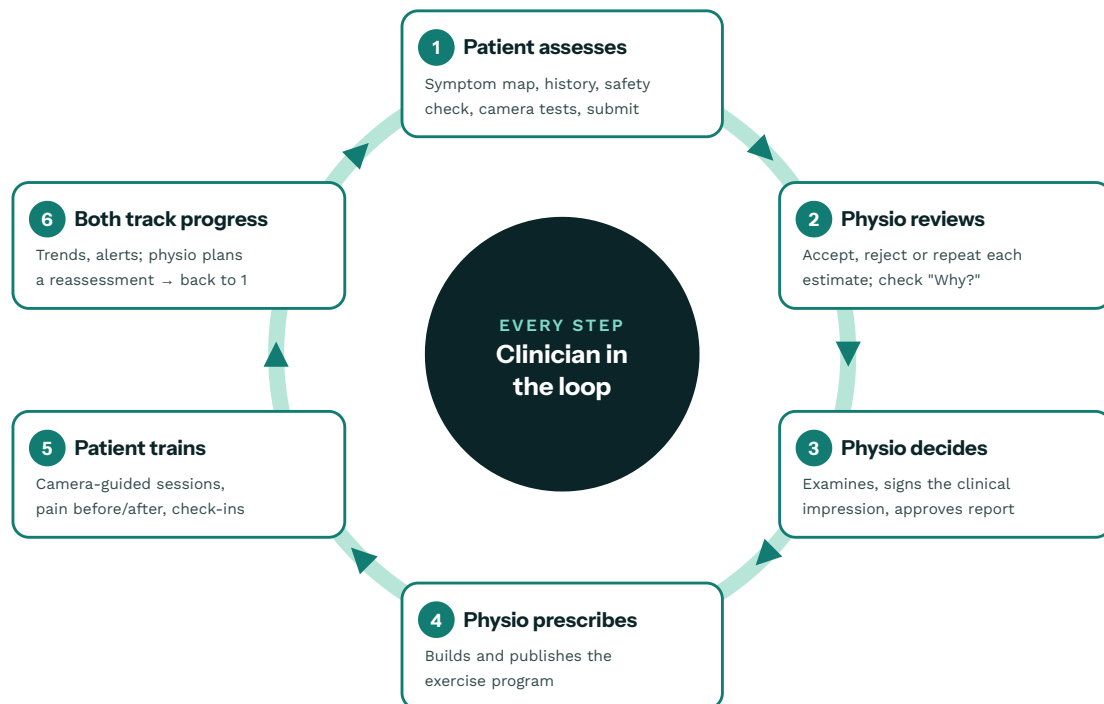
The app on one page

Everyone starts at the same website. The clinic decides who is a physiotherapist; everyone else gets the patient app. Both apps share records through the clinic server.



The care loop

Dheepika Lab follows one repeating loop. The patient provides answers and camera captures; the physiotherapist reviews them and makes every clinical decision.



What the patient does

Answers questions honestly, does the camera tests in a safe space, trains as prescribed, and checks in. Stops if pain is sharp or worsening.

What the physiotherapist does

Reviews every estimate, examines the patient, writes and signs the impression, approves the report and sets the program.

What the app does

Checks capture quality, withholds numbers it cannot measure, flags threshold crossings for review, and keeps a full audit log.

Four kinds of information, always kept apart

Throughout the app, each piece of information carries a label that says where it came from. Look for these tags:

TAG	MEANING
 Patient-reported	The patient's own answers: pain scores, symptom areas, history. Never changed by the app.
 Camera-estimated	A number calculated from body points seen by the camera, only when the capture passed the quality checks.
 Movement observation	An algorithmic observation : a camera estimate crossed a threshold set by the clinic. It is a flag for review, not a diagnosis.
 Clinician-measured	Measured or decided by the physiotherapist, for example with a goniometer, or a signed impression.

Accounts and signing in

Create a patient account

- 1 Open **physiovision-ai-eta.vercel.app** in Chrome or Safari on your phone.
- 2 Tap **Get started**. Enter your name, email and a password, then tap **Create account**.
- 3 Open the email from the clinic and tap the confirmation link. It brings you back to the app, signed in.
- 4 Complete the first-time setup (next section).

Link expired or already used? The sign-in screen says so. Go back to **I already have an account** and sign in, or request a new link.

Sign in and passwords

- **I already have an account** → email and password → **Sign in**.
- **Forgot password?** sends a reset email. The link opens a screen to choose a new password.
- **Sign out** is at the bottom of the menu. It removes this account's records from the device, which keeps a shared phone private. Unsent changes wait for your next sign-in.

Physiotherapist accounts

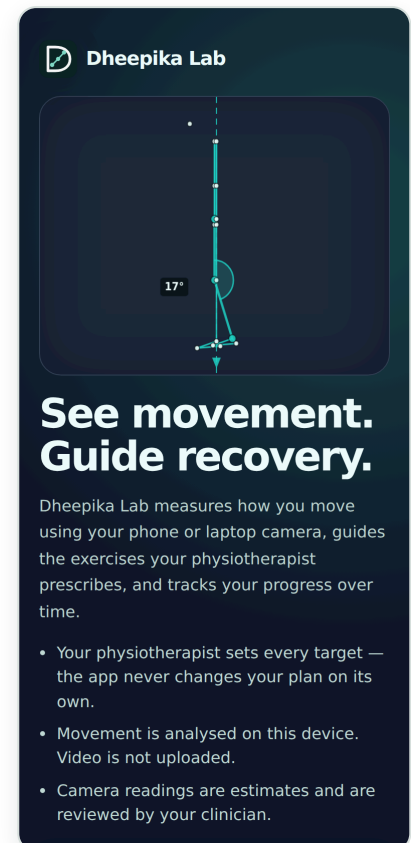
You cannot choose "physiotherapist" in the app. The clinic owner adds the physiotherapist's email to the clinic's list on the server. That person then signs up or signs in with that email and gets the physiotherapist app. An account created earlier is upgraded automatically.

Demo mode

Explore demo — patient and **Explore demo — physiotherapist** open the app with **SIMULATED** patients. A purple banner shows on every screen. Demo data stays on the device and is never sent to the clinic server. Use demo mode to learn the app or to train staff.

Tip: add it to your home screen

In the browser menu, choose **Add to Home screen** (Android) or **Share → Add to Home Screen** (iPhone). The app then opens like any other app.

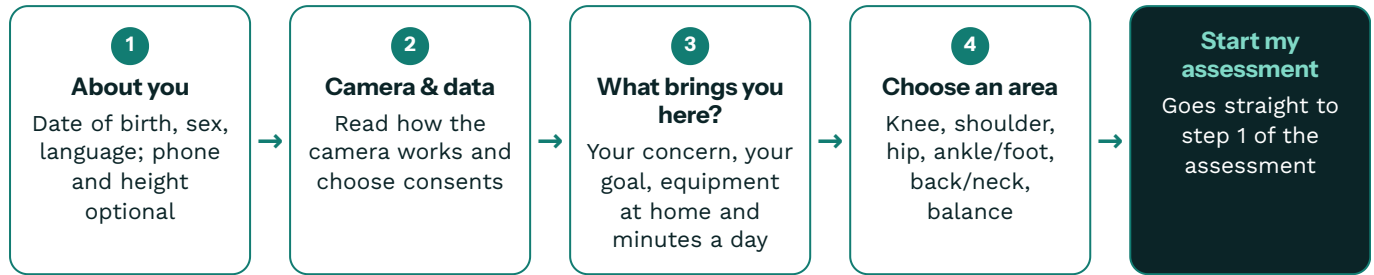


Welcome screen

Create account

First-time setup

A short setup runs once, right after your first sign-in. The progress bar shows "Step n of total".



Step 3 also asks what equipment you have at home and how many minutes a day you can give; your physiotherapist sees both.

Your consent choices

The first two are needed for movement assessment. The last two are optional. You can change every choice later in **Profile → Privacy & consent**.

CHOICE	WHAT IT MEANS	NEEDED?
On-device camera processing	The camera finds 33 body points and calculates joint angles on your phone. Video frames are processed in memory and thrown away.	Required
Store and share results	Your measurements and questionnaire answers are saved and shared with your physiotherapist. Every access and change is recorded in an audit log.	Required
Store a still image with each posture scan	Keeps one photo per posture view so your physiotherapist can check it. You can ask for your face to be covered.	Optional
Anonymised research use	Anonymised measurements may be used to test and improve accuracy.	Optional

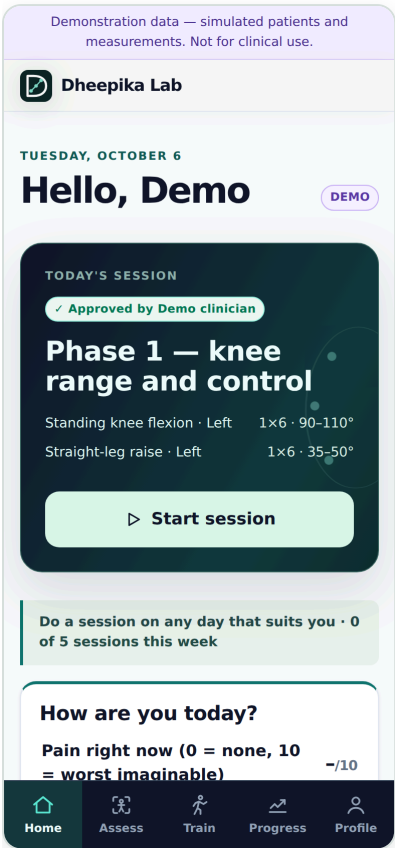
No video is stored. Only body-point coordinates and calculated measurements are saved, plus stills if you choose the optional image consent.

Which area first?

AREA	CAMERA TESTS AVAILABLE
Knee	Supported knee flexion (heel slide) · Five-times sit-to-stand · Double-leg squat (front view)
Shoulder	Active shoulder flexion (side view) · Active shoulder abduction (front view)
Hip	Active hip flexion, standing (side view) · Active hip abduction, standing (front view)
Ankle or foot	Knee-to-wall lunge (side view) · Double-leg heel raise (side view)
Back or neck	Standing forward bend (side view) · Standing side bend (front view) · Neck forward and backward movement (side view)
Balance and walking	Single-leg stance, eyes open (front view) · Marching on the spot (front view)

Every area also offers the optional standing posture scan. Your physiotherapist may change which tests you do. Later, use **Home → Assess another area**.

The Home screen



Home, with a program approved

What you see

- **Today's session:** your approved program, with each exercise, the side, sets × reps and the target range. Tap **Start session**.
- **This week:** sessions done out of the planned number. If you missed a day, just carry on with the next planned session; don't do extra to catch up.
- **How are you today?:** a daily check-in with your pain now (0–10) and an optional note. It goes to your physiotherapist. The app does not check it, so contact your clinic or urgent care if you feel unwell.
- **Next appointment, Assessment** status and **Recent sessions**.

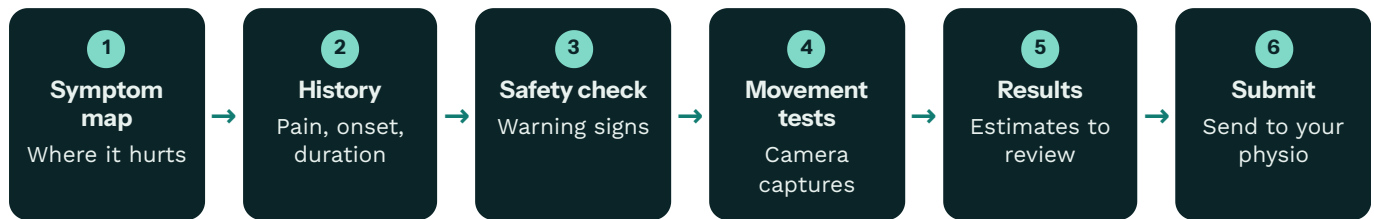
No program yet? Home says so and asks you to complete an assessment, so your physiotherapist can build your plan.

The bottom menu

TAB	USE IT TO
Home	See today's plan, check in, and see appointments
Assess	Start, resume or view an assessment
Train	See your full program and session history
Progress	View pain, range-of-motion and adherence charts
Profile	Change language, accessibility, consent, messages, export or delete

Doing an assessment

Six steps. You can stop at any time and tap **Resume assessment** later. The bar at the top shows where you are (for example "2/6").



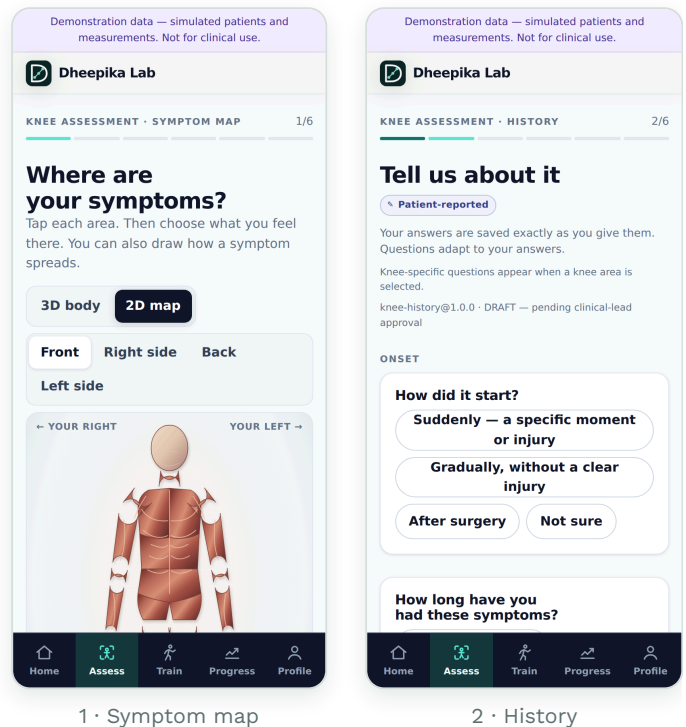
Step 1· Symptom map

- Use the **2D map** or the **3D body** (next page). Turn it with **Front / Right side / Back / Left side**. Labels show **your** left and right.
- Tap every area where you feel something, then choose what you feel there: pain, stiffness, weakness, numbness or tingling.
- For the knee, choose the exact spot: front / kneecap, inner side, outer side, back of knee or whole knee.
- If a symptom spreads, **draw its path** on the body.
- Prefer a list? Use **Choose areas from a list**.

The drawn body is an illustration to locate symptoms. It is not a camera image or a diagnosis.

Step 2· History

- Pain right now and worst in the last 24 hours (0–10).
- What it feels like, how long you have had it, how it started, what makes it worse, and whether it is constant.
- Questions adapt to your answers. They are saved **exactly as you give them**.



Step 3 · Safety check

Answer each question honestly. These questions help make sure exercise is appropriate right now. The answer decides what happens next:

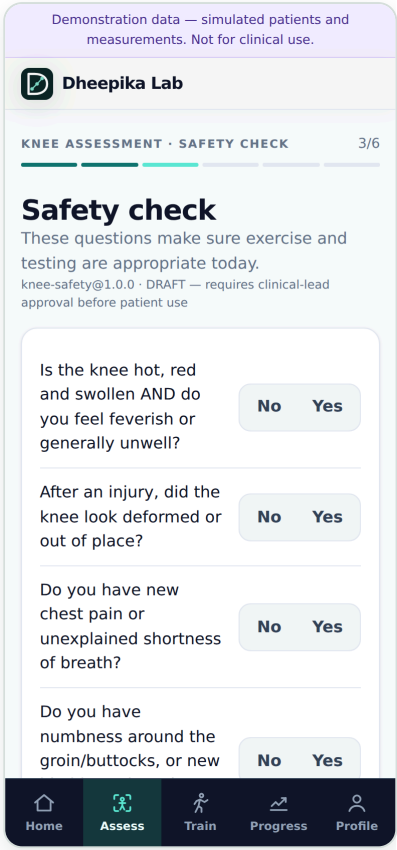
RESULT	WHAT HAPPENS
Seek urgent care	"Please seek urgent medical care now." Tests stop. Your physiotherapist sees a critical alert.
Speak with your physio first	Tests are paused until your physiotherapist reviews your safety answers.
Clear	Continue to the movement tests.

In an emergency, do not wait for the app. Call your local emergency number. Profile shows the clinic's emergency number.

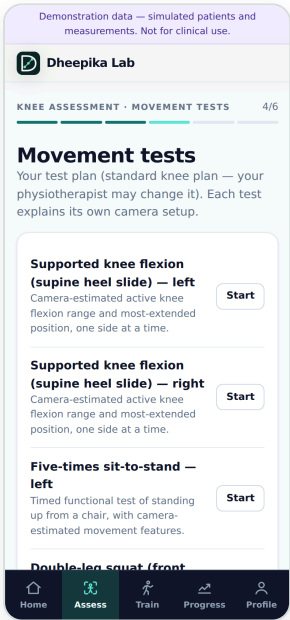
Step 4 · Movement tests

Your test plan is listed with a **Start** button for each test. Each test explains its own camera set-up and counts down before recording. A test shows ✓ **done** when the capture was valid, or ✗ **redo** when it was not. You can **Recapture** any test. The optional **static standing posture** scan (front, side and back) is at the bottom of the list.

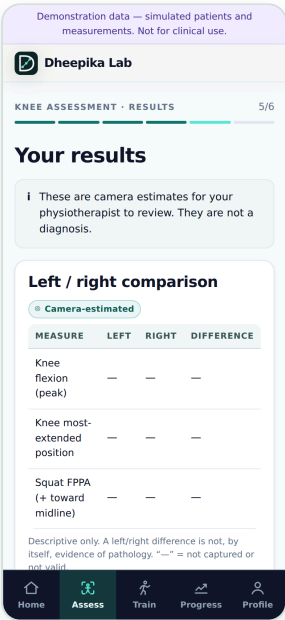
Before your first camera test, read the **Camera set-up guide** on the next page.



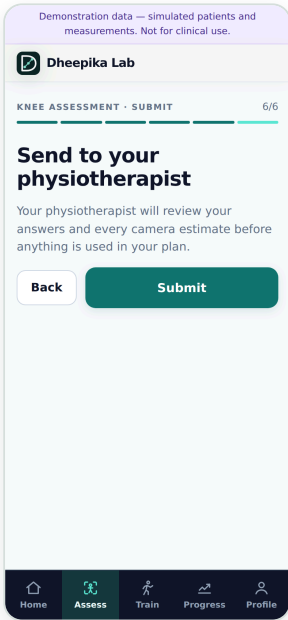
3 · Safety check



4 · Movement tests



5 · Results



6 · Submit

Step 5 · Results

Your camera estimates, such as a left/right comparison. A dash "—" means "not captured or not valid". These are estimates for your physiotherapist to review. They are not a diagnosis.

Step 6 · Submit

Tap **Submit to my physiotherapist**. Home then shows "Submitted — awaiting physiotherapist review", then "Reviewed by your physiotherapist" when they have checked it.

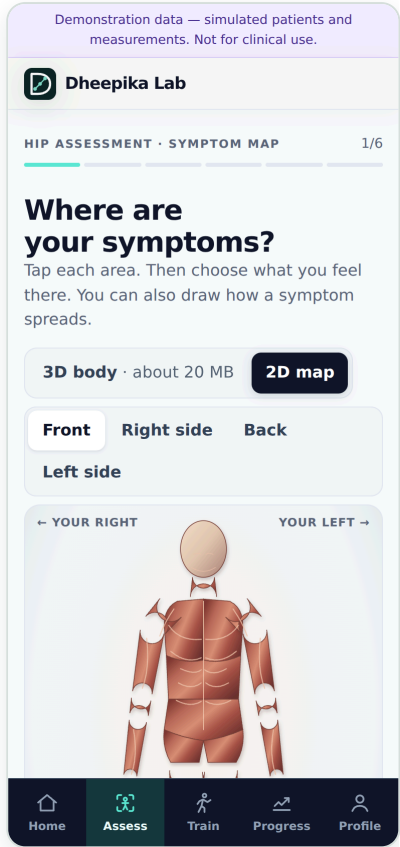
The body map: 2D or 3D

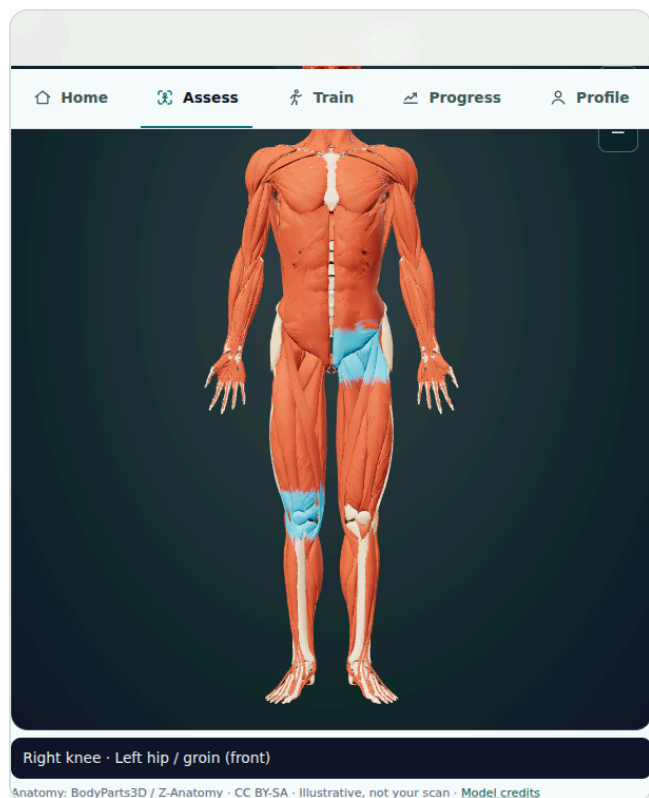
The symptom map comes in two forms. Both use the same body areas, so whatever you choose in one shows in the other and in your physiotherapist's review.

FORM	BEST FOR
2D map	Phones and slow connections. It opens first on phones and when data saver is on. Swipe sideways to turn the figure; scroll up and down normally.
3D body	Tap 3D body to download it once. Phones get the Light model (about 6 MB); laptops get Full detail (about 20 MB). You can switch between the two above the model. Drag to turn, use Front / Back / Left side / Right side , and + / - or pinch to zoom. Both models show the same body areas.

- **Tap where you feel it.** The area name appears under the model, for example "Right knee". You can also tap bones such as the kneecap or shin.
- Your choice stays the same when you turn the model. A calf chosen from the back is still the calf from the front.
- Prefer not to tap? Open **Choose areas from a list** under the map. It works with a screen reader and the keyboard (arrow keys turn the 3D model, + and - zoom).
- Some areas can only be seen from the side, such as the outer hip. The 2D map lists them as **Also selected, shown in other views**, with a link to that view.
- Drawing how a symptom spreads always uses the 2D map.
- If the 3D body cannot start or the phone runs short of graphics memory, the app switches to the 2D map and keeps your choices.

The 3D model is a licensed anatomy illustration (BodyParts3D / Z-Anatomy, CC BY-SA). It is not a scan of you, and choosing an area does not identify the painful tissue.





3D body on a laptop: right knee and left side selected (blue)

Phone upright,
level, on a stand

at the height the
test asks for

usually 2-3 m

Light on your front,
not behind you

Head to feet
in the picture

Fitted clothing,
knees and ankles visible

Nothing in front
of the body

Before recording, the app checks the set-up. Each line turns green when it passes. The capture waits until all of them pass.

The checks are smoothed, so a small sway does not flip "ready" on and off. The distance range adapts to your camera's resolution. On a sharp HD phone camera you can stand further back.



Posture scan tools

The standing posture scan takes front, side and back views, with a spoken instruction for each. Each view is a 3-second capture; the app uses the middle (median) value.

Two scan modes

Therapist-guided: the physiotherapist holds the device or uses a stand, 2–3 m away, and uses the rear camera. **Capture this view** unlocks only when the set-up checks pass.

Self-scan: the patient places the phone on a stand. The capture starts automatically when the checks pass and you hold still.

What the lines mean

- **Ruler:** percentage of ankle-to-nose height (0 = ankles, 100 = nose). It is a relative scale, not centimetres.
- **Plumb line:** dashed vertical line from the ankles.
- **Level lines:** head, shoulders and pelvis, with the measured angle.
- **A / L / R / P badge:** which view is being captured.

Display tools

TOOL	WHAT IT DOES
1x · 1.5x · 2.2x	Zooms the display only. The measurement is the same at every zoom.
Zoom to pain area	Zooms to the area the patient marked on the symptom map and outlines it as "Patient-reported". The button is off if no area was marked, or if that part of the body is not in view.
Region views	Shows four zoomed views at once (full body, head & neck, trunk, lower limb). They are crops of the same single camera, not extra cameras.
Colour · Grey · Negative	Changes the image tone for easier viewing. It shows the body surface only; it is not an X-ray .
Cover face	Hides the face on screen, and on saved stills.

Why a value shows "—"

A number appears only when every body point it needs is clearly visible. Otherwise the app withholds it and says why ("Waiting" means the set-up checks are not finished yet):

MESSAGE	WHAT TO DO
Hands or an object in front of the body	Arms relaxed by your sides; put the phone or any object away.
... not in view	Step back or re-aim the camera so the named joints are inside the picture.
... not clearly visible / covered	Loose clothing or shadows hide the joint. Wear fitted clothing and add light.
Not steady or clear enough during the capture	Stand still for the full 3 seconds and try again.
Could not be calculated in this position	Check you face the right way for this view.

A partly hidden body never gives a full-body result. If the phone or an arm covers part of the body, only the measurements that can be seen are shown.

Self-correction: before and after

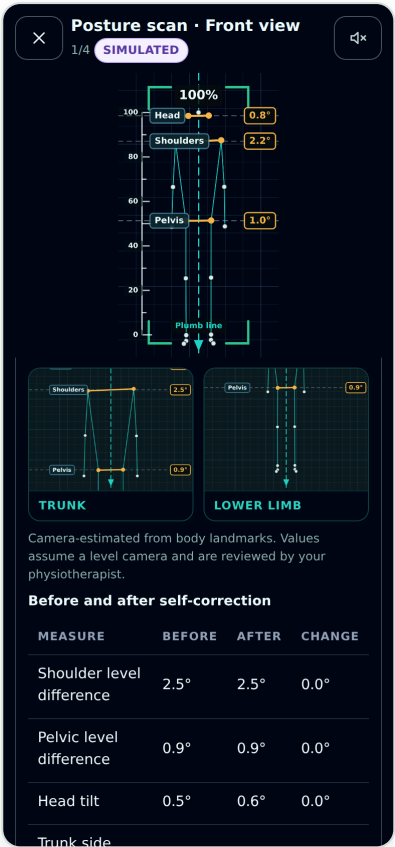
After a view is captured, you can tap **Try a 15-second self-correction**. A countdown starts and the app speaks the instruction: stand as tall and balanced as feels comfortable, let your shoulders settle, then hold still.

The view is then captured again and a table shows **Before, After** and **Change** for each measure.

- Only measures taken in **both** captures get a number. Anything else shows "—".
- This is for coaching. Your usual posture (the first capture) is the one saved for your physiotherapist.
- Tap **Self-correct again** to repeat, or **Retake** to start the view over.

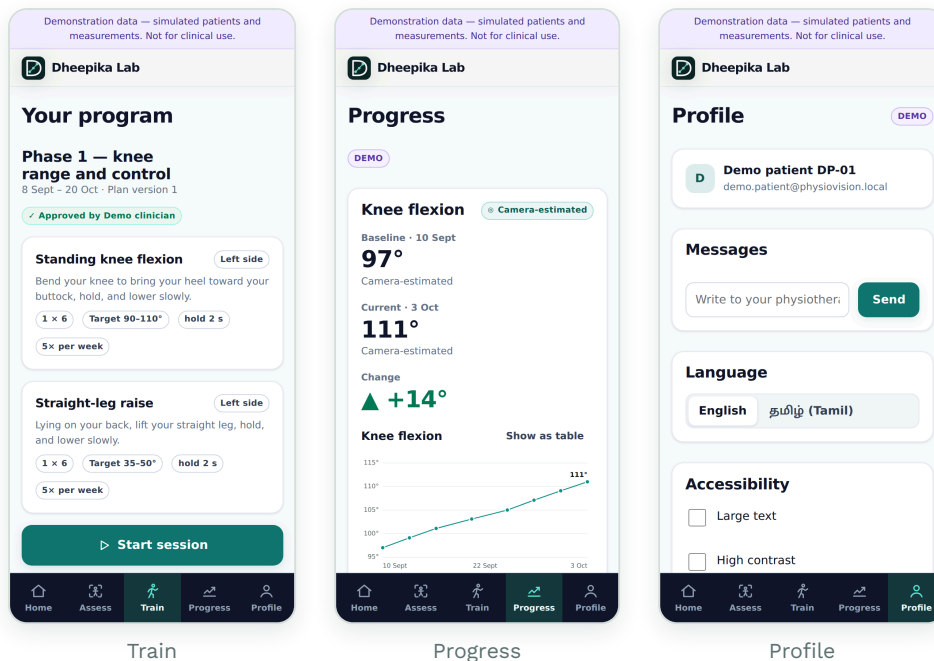
Spoken prompts

With the voice coach on, the app tells you how to stand for each view: face the camera; left side to the camera; right side to the camera; back to the camera.

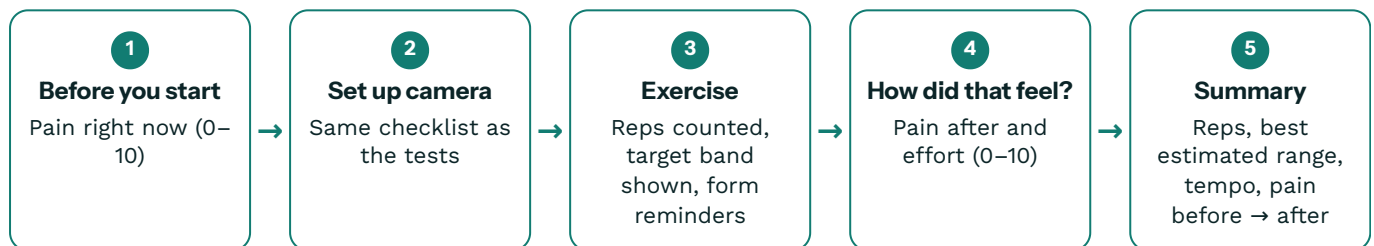


Before / after table

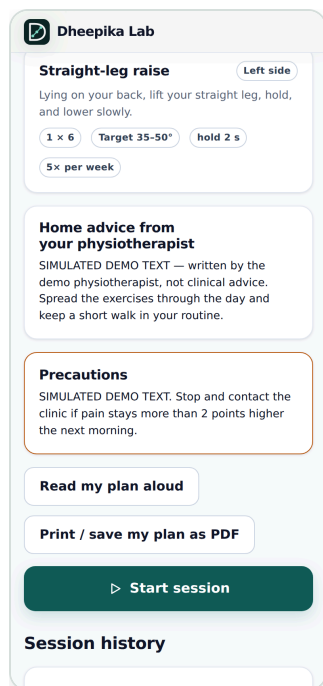
Training, progress and profile



An exercise session



- The **Train** tab shows the program, who approved it, sets × reps, target range, hold time, how often per week, and your session history. Exercises that do not need the camera are listed under **Also in your plan**.
- If your pain rises past the limit your physiotherapist set, the session pauses and your physiotherapist is told. **Stop if pain is sharp or getting worse.**
- If a session is interrupted, the progress so far is saved.



Home advice, precautions, read aloud, print

Advice, precautions and your plan on paper

- **Home advice from your physiotherapist** and **Precautions** appear under your exercises when your physiotherapist writes them. They are in your physiotherapist's own words.
- **Read my plan aloud** reads the exercises, sets, holds, advice and precautions using your phone's own voice. Nothing is sent anywhere. Tap again to stop.
- **Print / save my plan as PDF** prints just the plan (not your history).
- Exercises without the camera may show **Tips** and **Avoid** lines, written and approved by your physiotherapist.

Progress

Charts of pain (0–10), range of motion and adherence, from baseline to now. **Camera-estimated** values (session peaks) and **clinician-measured** values (goniometer) are drawn differently, so you can tell them apart. Tap **Show as table** for exact numbers.

Profile

- Language (English; Tamil is a draft awaiting review).
- Large text, high contrast, reduced motion, voice coach, captions, vibration.
- Privacy & consent; the clinic's emergency number; messages to your physiotherapist; links to **How it works & accuracy** and the **Privacy notice (draft)**.
- **Export my data**, or **Delete my account and data** (type DELETE to confirm). Deleting removes your records from the clinic server and the device.

My goals

Progress → **My goals**. Write what you would like to be able to do again, in your own words, for example "Climb the stairs at home without stopping". Your physiotherapist goes through it with you and marks it agreed.

- 1 Type the goal under **Add a goal** and press **Save goal**.
 - 2 From time to time, choose how close you feel to it today, from 0 (not at all) to 10 (there). Add a note if you like.
 - 3 Press **Save rating**. Your physiotherapist sees every rating with its date.
- The app never writes or suggests a goal for you.
 - Your rating is your own answer. It is **not** calculated from camera measurements and is shown separately from them.
 - Saved ratings are kept as given; a new rating is added, never changed.

My goals

Patient-reported

What would you like to be able to do? Write it in your own words; your physiotherapist will go through it with you.

"Climb the stairs at home without stopping"

Not yet discussed with your physiotherapist · last rated 3/10 on 7 Oct

How close do you feel to this goal today? (0 = not at all, 10 = there)

0	1	2	3
4	5	6	7
8	9	10	

Note (optional)

Save rating

Add a goal

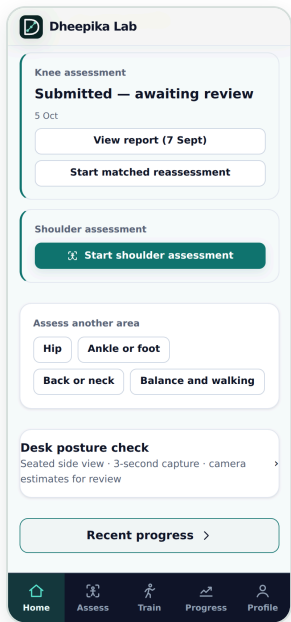
e.g. Walk to the temple without stop

Save goal

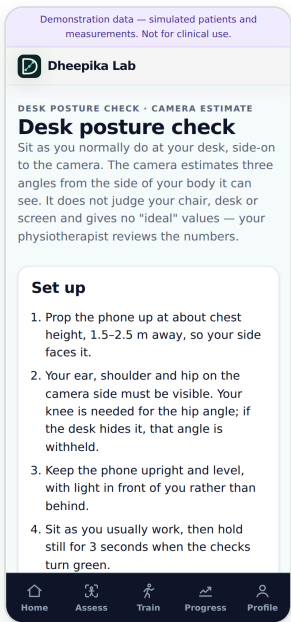
Progress: My goals

Desk posture check

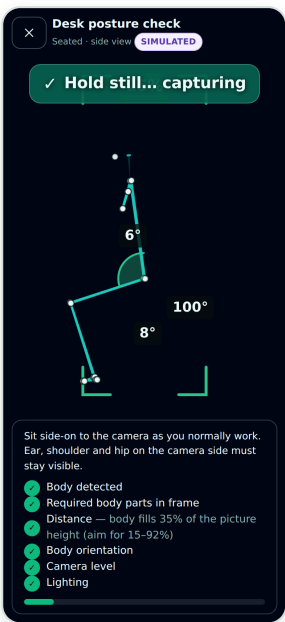
A short, seated check of how you sit at your desk, seen from the side. It records three angles for your physiotherapist. It gives no "ideal" values, scores or advice about your chair or screen.



Home: Desk posture check



Set up and what is measured



Checks, then a 3-second hold

How to do it

- 1 Home → **Desk posture check** → read the set-up → **Start camera**.
- 2 Prop the phone at about chest height, 1.5–2.5 m away, with your **side** towards it. Keep it upright and level, with light in front of you.
- 3 Sit as you normally work. When every check turns green, hold still for 3 seconds.
- 4 Read the results and tap **Save for my physiotherapist**, or **Retake**.

What it measures

MEASURE	HOW
Ear–shoulder line	Angle of the ear-to-shoulder line from vertical. Positive means the ear is in front of the shoulder. A 2D proxy, not a craniovertebral angle.
Trunk lean	Angle of the shoulder-to-hip line from vertical. Positive means leaning forward.
Hip angle	Angle at the hip between trunk and thigh. Withheld if the desk hides your knee.

Reading the results

- Each value is the **median** of the frames in the 3-second hold, with the number of frames, the variation (SD) and the body-point confidence.
- A value shows **"—" Withheld** with the reason when the needed body points were hidden, the facing direction was unclear, or the reading changed too much. Fix the set-up and retake.
- Saved values go to your physiotherapist as **camera estimates waiting for review**. They are not compared with any "normal" value and are not a diagnosis.
- No video or picture is kept.

Clothing, the chair back, and the camera's height and angle all change these numbers. Use the same set-up each time if your physiotherapist asks you to repeat it.

Demonstration data — simulated patients and measurements. Not for clinical use.

 **Dheepika Lab**

DESK POSTURE CHECK · CAMERA ESTIMATE

Desk posture check results

SIMULATED

Ear-shoulder line (seated) 7.4°

angle of the ear-to-shoulder line from vertical; positive = ear forward of the shoulder. A 2D proxy, not a craniocervical angle.

Median of 182 frames · SD 0.8° · landmark confidence 97%

Trunk lean (seated) 8.0°

angle of the shoulder-to-hip line from vertical; positive = leaning forward.

Median of 182 frames · SD 0.2° · landmark confidence 97%

Hip angle (seated) 100.1°

angle at the hip between the trunk and the thigh (shoulder-hip-knee).

Median of 182 frames · SD 0.3° · landmark confidence 97%

i Camera estimates only. Saving sends them to your physiotherapist for review; they are not compared with any "normal" value.

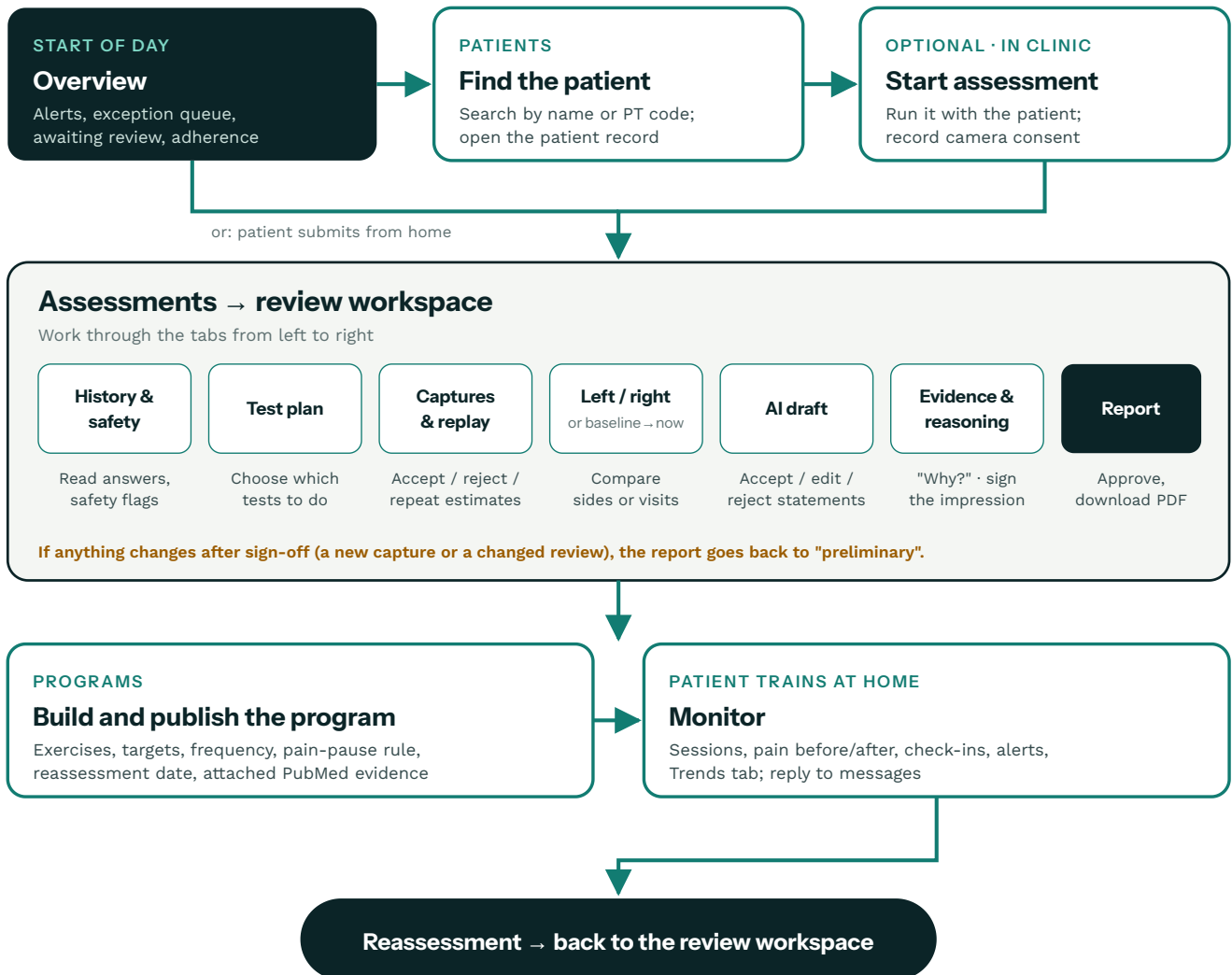
Retake

Save for my physiotherapist

Results (simulated demo)

The physiotherapist's map

A typical path from a new patient to a reassessment. Use a laptop or tablet for review work. On a phone the menu shows Overview, Patients, Assessments, Schedule and **More** for everything else.



Overview and patients

Demonstration data — simulated patients and measurements. Not for clinical use.

Dheepika Lab

- Overview**
- Patients
- Assessments
- Schedule
- Billing
- Programs
- Library
- Research
- Anatomy
- Analytics
- Settings
- Sign out

DHEEPIKA LAB

Overview

Demo clinician · Physiotherapist (simulation)

Active patients

1

5 total

Assessments awaiting review

5

Requires clinician review >

Sessions (7 days)

2

Open alerts

5

1 critical

Exception queue

Demo patient DP-01
4 of 10 planned sessions in the last 14 days
plan Phase 1 — knee range and control v1, 5/week

Demo patient DP-01
Submitted assessment waiting 2 day(s) for review
knee assessment 2026-10-05

Rules (9 unreviewed)

rule threshold unreviewed

rule threshold unreviewed

Reported symptom changes & alerts

Demo patient DP-03
Red flag — urgent · urgent: 7 Oct
calf (knee-safety@1.0.0)

Awaiting review [View all](#)

Demo patient DP-01
5 Oct, 09:14 am · 11
pending estimates

Program adherence (28 days)

Demo patient DP-01

40% **low**

Overview (demo data)

Overview

- **Counts:** active patients, assessments awaiting review, sessions in the last 7 days, and open alerts (critical ones counted separately).
- **Exception queue:** patients who need attention, such as low adherence. It also flags **overdue work:** an assessment or safety hold waiting for review, a reassessment that is due, or a plan paused by the pain rule that nobody has resumed. Each rule's threshold is set by the clinic and shows "unreviewed" until a clinician reviews it.
- **Reported symptom changes & alerts:** safety flags, such as a red flag that needs urgent care.
- **Awaiting review** and **Program adherence (28 days)**.

Patients directory

Every patient of the clinic, from every device, with a **PT code** (for example PT-3F9A21), age and sex, main complaint and registration date. Search by name, **phone**, PT code or complaint, and filter by body region. The four cards at the top (**Total patients**, **Visits attended**, **Active programs**, **Completed rehab**) are counted from records; tap one to filter the list. Long lists show 20 per page. **Export CSV** saves the rows you are looking at and is recorded in the audit log; **Print** prints the list. Use **Start assessment** to run an assessment with the patient in the clinic.

Dheepika Lab

Overview

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Sign out

Demonstration data — simulated patients and measurements. Not for clinical use.

Registered patient directory

+ Register patient

Export CSV

Print

Total patients

5

All registered

Visits attended

3

Marked attended in the schedule

Active programs

1

Show patients

Completed rehab

0

Discharged or course completed

Search by name, phone, patient ID or complaint...

All body regions

All

On a program

Completed

Needs review

Alerts

Low adherence

PATIENT ID	PATIENT · AGE / SEX	PRESENTING COMPLAINT (PATIENT'S WORDS)	REGISTERED	PROGRAM	ADHERENCE	ALERT
PT-4EBE04	DP Demo patient DP-01 34 y · male	-	28 Aug 2026	Phase 1 — knee range and control last session 4 Oct	40%	! 1
PT-BB81CF	DP Demo patient DP-02 58 y · female	-	28 Aug 2026	None	-	! 1
PT-C45913	DP Demo patient DP-03 45 y · female	-	28 Aug 2026	None	-	! 1
PT-67128C	DP Demo patient DP-	-	28 Aug 2026	None	-	! 1

Patients directory

The patient record

Header buttons: **Start assessment**, **Build program**, **Discharge** and **App QR**. The QR code opens the app's sign-in page on the patient's own phone; it contains only the app address. The header also shows the patient's equipment at home and daily time, as they reported them.

TAB	CONTENTS
Overview	Summary, current program, alerts
Assessment	Assessments for this patient and their status
Movement measurements	Camera estimates and clinician measurements, each with its source
Patient-reported outcomes	Questionnaire answers and pain scores
Programs	Current and past programs, with versions
Trends	Changes over time
Devices & export	Shared step and walking data, and data export
Session history · Progress	Every exercise session; progress charts
Goals	Goals in the patient's own words and the patient's own 0–10 ratings (new)
Clinical notes	SOAP notes with quoted, source-labelled measurements; corrections are new notes (new)
Letters	Referral and progress-update letters: draft, sign, new version (new)

- web- and phone-hostings. This creates a record only, the patient can create their own app login later from the app QR. Record the complaint in the patient's own words, or leave it blank.
- | Full name | Mobile (optional) | Date of birth (optional) | Sex (optional) | Complaint in the patient's words (optional) |
|-----------|-------------------|--------------------------|----------------|---|
| Lakshmi R | 98765 43210 | mm/dd/yyyy | Female | knee pain on stairs |
- Register patient

Dheepika Lab

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DHEEPIKA LAB

Overview

Demo clinician · Physiotherapist (simulation)

+ Programs

Active patients

1

5 total

Assessments awaiting review

5

Requires clinician review >

Sessions (7 days)

2

Open alerts

5

1 critical

Exception queue

Rules (9 unreviewed)

Demo patient DP-01

4 of 10 planned sessions in the last 14 days

plan Phase 1 — knee range and control v1, 5/week

rule threshold unreviewed

Demo patient DP-01

Submitted assessment waiting 2 day(s) for review

knee assessment 2026-10-05

rule threshold unreviewed

Reported symptom changes & alerts

Demo patient DP-03

Red flag — urgent - urgent: 7 Oct

calf (knee-safety@1.0.0)

Awaiting review [View all](#)

DP

Demo patient DP-01

5 Oct, 09:14 am · 11

pending estimates

Program adherence (28 days)

Demo patient DP-01

40%

low

23 / 43

Goals, structured notes and letters

Goals tab. Record a goal in the patient's words when you set it together, or mark the patient's own goal as agreed. Mark it achieved or withdrawn when the time comes. Ratings are the patient's own and are not changed.

"Return to weekend badminton"
active

Set 7 Oct · agreed 7 Oct

↗ Patient-reported Not rated yet

Mark achieved Withdraw

Goals tab

Clinical notes tab. Write the note under **S — Subjective**, **O — Objective**, **A — Assessment**, **P — Plan**; empty sections are left out. Open **Quote measurements** to copy values into the note. Each quoted value keeps its source label (patient-reported, camera estimate, algorithmic observation or clinician finding) with view, confidence, review state and version, frozen as it was. Withheld camera values are quoted as **withheld**, never as a number. Notes are not edited: **Correct this note** saves a new note and keeps the original under **Earlier versions**.

Demo clinician
7 Oct, 09:13 am

S — Subjective
Stairs painful going down, easier going up.

O — Objective
Squat repeated in clinic; see quoted camera values.

A — Assessment
Improving; tolerance to stairs better than last week.

P — Plan
Continue programme; reassess in two weeks.

Quoted measurements

- Squat depth (hip descent): 30% [Camera estimate; view anterior; confidence 94%, not yet reviewed; version pv-knee-1.1.0; 2026-10-05]

Correct this note

A SOAP note with a quoted camera estimate

Letters tab. **New letter** → choose **Progress update** or **Referral**, add the recipient, write the letter and quote any values. **Save draft** keeps it editable; **Sign letter** makes it final. To change a signed letter, use **New version**; the signed one stays as it was. **Download text** includes a fixed paragraph on the limits of camera estimates. Drafts are marked "DRAFT — not signed".

Progress update to Orthopaedic outpatient clinic, City Hospital
Signed 7 Oct

Thank you for seeing this patient. Knee pain on stairs has improved only slightly after six weeks of home exercise. I would value your opinion.

- Daily check-in: 2/10 — "Demo: stairs easier this week" [Patient-reported; 2026-10-06]

New version Download text

A signed letter

Reviewing an assessment

Open **Assessments**, pick a submitted assessment, and work through the seven tabs.

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Demonstration data — simulated patients and measurements. Not for clinical use.

KNEE ASSESSMENT · SUBMITTED

Demo patient DP-02 DEMO

58 y · started 5 Oct, 01:29 pm · submitted 5 Oct, 02:29 pm

History & safety

Test plan

Captures & replay

Left / right

AI draft

Evidence & reasoning

Report

Showing the latest capture per test (4 captures in total, earlier attempts retained). Accepting a camera estimate records that you reviewed it; it remains labelled camera-estimated.

Supported knee flexion (supine heel slide) — left

knee_supported_flexion@1.1.0 · pv-knee-1.1.0 · synthetic-skeleton 1.0.0 · view lateral_left · 5 Oct, 01:29 pm

Camera-estimated

SIMULATED

valid capture

Knee flexion (peak)

124.1°

per rep: 124.1, 123.9, 123.8

Knee extension (most-extended position)

2.9°

0° = straight. Higher = knee did not fully straighten. Hyperextension cannot be measured by this 2D method.

Quality: coverage 100% · mean landmark confidence 0.97 · 3 valid of 3 attempts · 30 fps

Show synchronized replay

Reference measurement for validation (0 recorded)

Clinician-measured

knee flexion peak

pending

Accept

Reject

Repeat

knee extension position

pending

Accept

Reject

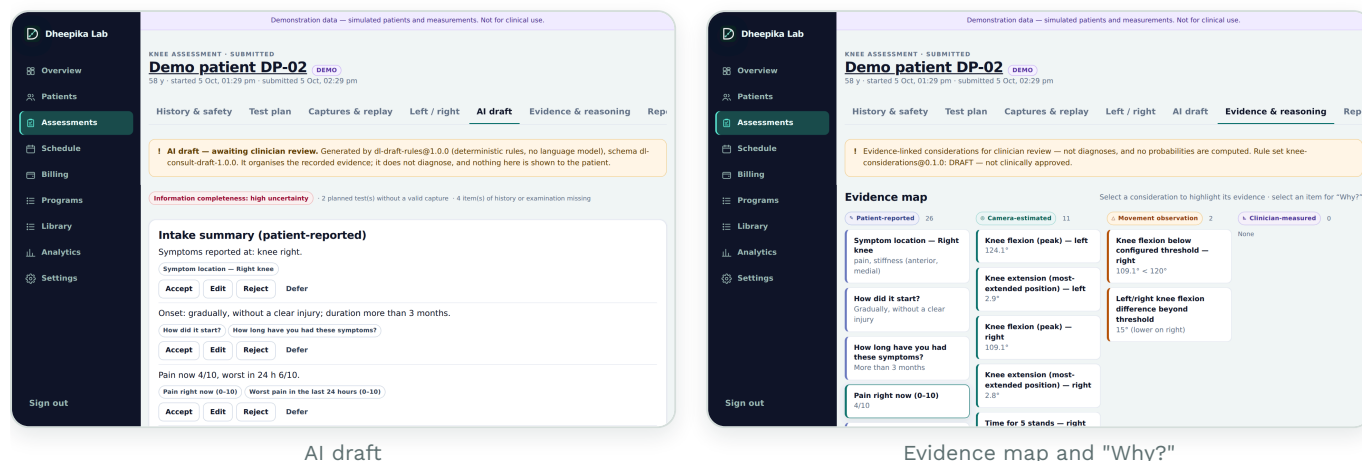
Repeat

Supported knee flexion (supine heel slide) — right

Captures & replay: estimate, version, view, quality line and review buttons

TAB	WHAT YOU DO THERE
History & safety	Read the symptom map, radiation path and the original answers. Safety flags are shown at the top. A safety hold withholds routine automated conclusions.
Test plan	Choose which tests the patient should do. The patient's app follows your plan.
Captures & replay	For each test, see the latest valid capture with its protocol version, camera view, quality (coverage, confidence, valid attempts, frame rate) and a synchronised replay. Accept , Reject or Repeat each estimate. Enter a reference measurement (for example a goniometer reading) for validation. The posture board shows each view with region zooms, and a grey or negative tone.
Left / right	Compare sides. For a reassessment, this tab shows baseline → current. A difference is descriptive only, not evidence of pathology by itself.
AI draft	A structured draft (see next page) to accept, edit, reject or defer, statement by statement.
Evidence & reasoning	The evidence map and "Why?" view. Record your examination findings and the clinical impression .
Report	Preview, approve and download the report.

AI draft, "Why?" and the report



AI draft

The draft is built from fixed, versioned rules, not a language model. It organises the recorded evidence into five sections: intake summary (patient-reported), observations (camera-estimated / algorithmic), safety flags, contradictions to clarify, and considerations. Every statement shows the evidence it cites. For each one, choose **Accept**, **Edit**, **Reject** or **Defer**. The original proposal is kept next to your decision. An **information completeness** banner lists missing tests or history. Nothing in the draft is shown to the patient.

Evidence & reasoning: the "Why?" view

The evidence map has four columns: **Patient-reported**, **Camera-estimated**, **Movement observation**, and **Clinician-measured**. Select an item to see **why** it is there: the measurement, camera view, confidence, calculation, rule version and limitations. Considerations are evidence-linked prompts for your review. They are not diagnoses, and no probabilities are computed.

The report



The report uses the latest valid capture for each test and view. If a capture or review changes after approval, the report goes back to **preliminary** until you approve it again. Use **Print view** to print.

Programs, library and evidence

Program builder

Building a program

- 1 Open **Programs** → **new**. Choose the patient, program title, start and end dates.
- 2 Add camera-tracked exercises. For each one, set the side, sets, repetitions, range-of-motion target (min–max °), hold, rest, minimum rep duration (tempo), frequency per week and special instructions.
- 3 Set the **pain-pause rule**: pause the session if patient-reported pain reaches a level, or rises by a set number of points above the pre-session score. Leave it blank to switch it off.
- 4 Add **library exercises** that do not need the camera (sets, reps, hold or duration, frequency).
- 5 Set the **reassessment** date and pauses.
- 6 Write **Home advice for the patient** (for example ice or heat, pacing) and **Precautions** (when to stop, what to avoid). The patient sees these words exactly, can hear them read aloud and can print them. Leave blank if not needed.
- 7 Optional: **Search PubMed** and **Attach to plan** supporting articles. Records are shown exactly as PubMed returns them; the app never writes citations itself.
- 8 Publish. The patient sees it on Home as "Approved by ...". Editing creates a new version with a list of changes from the previous one.

Library

The **Exercise library** holds exercises that do not need the camera. Each one is marked **Home after prescription** or **In clinic only**; in-clinic-only items cannot be added to a home plan, and changing the level creates a new version for review. The program builder flags items that need equipment the patient did not list. The **ROM measurement guide** (button on the Library page) shows goniometer placement for each joint the app measures, without normal ranges.

Open an exercise and use **Edit cues and common mistakes** to add short coaching lines (one per line). Saving creates a **draft version**; patients see the new tips only after it is reviewed and approved.

Research and the anatomy explorer

Dheepika Lab

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- Patients
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- Schedule
- Billing
- Programs
- Library
- Research**
- Anatomy
- Analytics
- Settings

Sign out

knee osteoarthritis exercise therapy patellofemoral pain exercise anterior cruciate ligament reconstruction rehabilitation

chronic low back pain exercise neck pain exercise therapy rotator cuff related shoulder pain exercise

frozen shoulder physiotherapy lateral elbow tendinopathy plantar heel pain ankle sprain rehabilitation

falls prevention exercise older adults office workers neck pain ergonomics

To attach an appraised paper to a treatment plan, open the Evidence section in the planner. The record count is not a clinical consensus.

2,229 records · first 20 by PubMed relevance

Query sent: (knee osteoarthritis exercise therapy) AND free full text[sb]

Comparative efficacy and safety of exercise modalities in knee osteoarthritis: systematic review and network meta-analysis.

BMJ (Clinical research ed.) · 2025 · PMID 41093618 · Comparative Study, Journal Article, Network Meta-Analysis, Systematic Review

[Hide abstract](#) [Free full text \(PubMed Central\)](#) [DOI 10.1136/bmj-2025-085242](#)

OBJECTIVE: To assess the efficacy and safety of various exercise modalities as therapeutic interventions for managing knee osteoarthritis.

DESIGN: Systematic review with network meta-analysis.

DATA SOURCES: PubMed, Embase, Cochrane Library, Web of Science, CINAHL, PsycINFO, AMED, PEDro, Scopus, ClinicalTrials.gov, ICTRP, and ClinicalTrialsRegister.eu from database inception to August 2024.

ELIGIBILITY CRITERIA FOR SELECTING STUDIES: Randomised controlled trials comparing different exercise modalities, including aerobic exercise, flexibility exercise, mind-body exercise, neuromotor exercise, strengthening exercise, mixed exercise, and control group for patients with knee osteoarthritis.

MAIN OUTCOME MEASURES: Primary outcomes included pain, function, gait performance, and quality of life, assessed at short term (four weeks), mid-term (12 weeks), and long term (24 weeks) follow-up. When exact time points were unavailable, data from adjacent time windows were used.

RESULTS: 217 randomised controlled trials involving 15 684 participants were included. Moderate certainty evidence showed that, compared with the control group, aerobic exercise probably results in large improvements in pain at short term (standardised mean difference -1.10, 95% confidence interval -1.68 to -0.52) and mid-term follow-up (-1.19, -1.59 to -0.79), function at mid-term (1.78, 1.05 to 2.51), gait performance at mid-term (0.85, 0.55 to 1.14), and quality of life at short term (1.53, 0.47 to 2.59). Mind-body exercise probably results in a large increase in function at short term follow-up (0.88, 0.03 to 1.73; moderate certainty), while neuromotor exercise probably results in a large increase in gait performance at short term follow-up (1.04, 0.51 to 1.57; moderate certainty). Strengthening (0.86, 0.53 to 1.18) and mixed exercise (1.07, 0.68 to 1.46) probably result in a large increase in function at mid-term follow-up, all with moderate certainty evidence. Regarding long term follow-up, flexibility exercise may result in a large reduction in

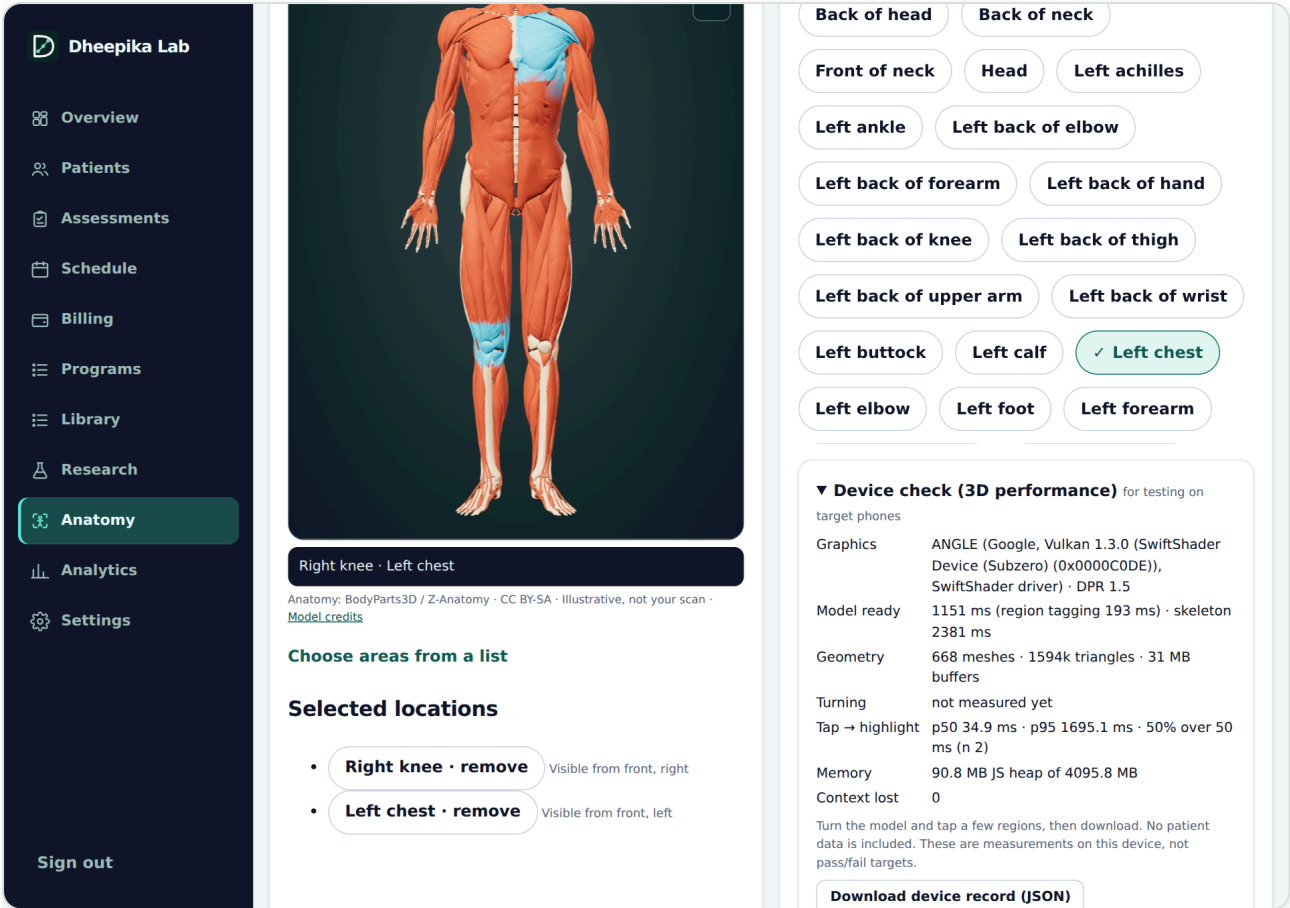
Research: live PubMed search with filters and the abstract opened

Research

- 1 Open **Research**. Type search terms or tap a **quick topic** (an editable starting point, not a recommendation).
- 2 Optional PubMed filters: **Systematic reviews**, **Guidelines**, **Randomised controlled trials**, **Free full text**, **Last 5 years**. The exact query sent is shown.
- 3 For each record: title, journal, year, PMID and publication types. **Show abstract** loads the abstract exactly as published. **Free full text (PubMed Central)** and **DOI** links open the paper.
- 4 To attach a paper to a patient's plan, use the Evidence section in the program builder (**Open treatment planner**).

The app does not summarise, grade or rank papers; appraise them yourself. Only your search terms are sent to PubMed. **Never enter names or patient details.**

Anatomy explorer

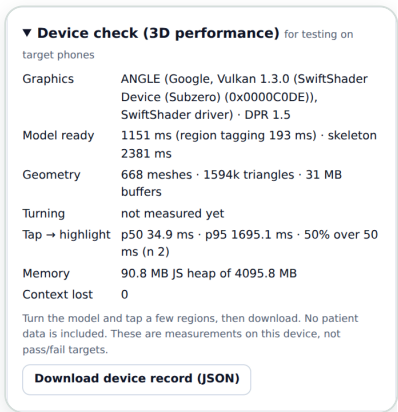


Anatomy: 3D atlas, region index, selected locations and the device check

- A reference atlas using the same body areas as the patient's symptom map. Tap the 3D model, use the 2D map, or search the **Region index** (for example "left knee").
- Selected locations list which views show them. A selected location never establishes the painful tissue or a condition.
- Links to the **Exercise library**, **ROM measurement guide** and **Research**.

Device check (for testers)

Opens under the region index. With the 3D body open, turn the model and tap a few areas. It then shows what the 3D atlas costs on **this** device: graphics chip, time to load, frame time while turning, tap-to-highlight time, memory, and whether the graphics context was lost. **Download device record (JSON)** saves it for the phone test log (see the testers' chapter). It contains no patient data and is a measurement, not a pass/fail target.



Device check panel

Schedule and billing

Run the clinic day in the app: book visits, mark who attended, and keep courses, payments and expenses in one place. Every figure is a total of what staff record; nothing is estimated.

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Demonstration data — simulated patients and measurements. Not for clinical use.

CLINIC

Schedule

Book visits, record attendance and see each day's agenda. Session counts on treatment courses come from attendance recorded here.

Billing

October 2026

MON

TUE

WED

THU

FRI

SAT

SUN

1

2

3

4

5

6

7

8

9

10

11

12

13

14

15

16

17

18

19

20

21

22

23

24

25

26

27

28

29

30

31

Wednesday 7 October

Nothing booked for this day.

The WhatsApp reminder contains only the date, time, clinic and staff name. It opens WhatsApp on this device; nothing is sent until you press send there.

Book a visit

Patient

Date

Time

Staff

Type

Select...

10/07/2026

09:00

Demo clinician

Treatment ses...

Treatment course

Note (optional)

No active course

Book

Walk-in now (attended)

New patient? Register them here

Schedule: month calendar and the selected day's visits

Schedule

- 1 Pick a day on the calendar. The number under a date is how many visits are booked.
- 2 Under **Book a visit** choose the patient, date, time, staff member, visit type and, for treatment sessions, the patient's course. A second booking for the same staff member at the same time is blocked.
- 3 After the visit, mark it **Attended** or **Missed** (or **Cancel**). **Undo** reverses a mistake.
- 4 Someone arrives without a booking? Pick them and press **Walk-in now (attended)**: a visit at the current time is recorded as attended. A new person can be registered right there with **New patient? Register them here**.
- 5 If the patient has a phone number, **WhatsApp reminder** opens WhatsApp with a message carrying only the date, time, clinic and staff name. Nothing is sent until you press send in WhatsApp.

Add or pause staff under **Staff**. Adding a name there does not create a login; physiotherapist logins are added by the clinic owner.

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Demonstration data — simulated patients and measurements. Not for clinical use.

CLINIC

Billing

Treatment courses, payments, expenses and monthly statements. Figures are totals of what staff record here.

Schedule

Received in Oct 26

₹0.00

Expenses in Oct 26

₹15,000.00

Net in Oct 26

-₹15,000.00

Outstanding dues

₹3,200.00

1 patient(s)

Treatment courses

Export CSV

Attended and missed are counted from visits marked on the schedule. Courses do not show recovery; that is judged by the physiotherapist from assessments.

PATIENT	COURSE	SESSIONS	AGREED FEE	STATUS
Demo patient DP-01 DEMO	Knee rehabilitation (demo) from 2026-09-22	3/8 attended · 1 missed · 2 booked	₹6,400.00	active

New course

Patient

Course name

Planned sessions

Agreed fee (₹)

Start date

Select...

e.g. Knee rehabilita

10/06/2026

Add course

Billing: this month, courses, dues, payments, expenses and statement

Billing

PART	WHAT IT DOES
Treatment courses	The sessions and fee agreed with the patient. Attended and missed counts come from the schedule. Mark a course Complete or Stop .
Outstanding dues	For each patient: agreed fees minus payments received.
Payments received	Amount in rupees, method (cash, UPI, card, bank transfer), date and an optional receipt or UPI reference.
Expenses	Rent, salaries, utilities, equipment, supplies, software and other costs.
Monthly statement	Money received, expenses and net for each of the last 12 months, as a chart and a table.
Export CSV	Courses, payments, expenses or the statement as a spreadsheet file. Each export is recorded in the audit log.

Courses do not show "recovery". Progress is judged by the physiotherapist from assessments. Only physiotherapist accounts can record courses, payments and expenses.

Discharge summary

Dheepika Lab

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Sign out

Demonstration data — simulated patients and measurements. Not for clinical use.

DISCHARGE

Demo patient DP-01 DEMO

PT-BE24A4

Back to patient

Discharge summary Preliminary — not signed

Attendance, pain scores and measurements are filled in from the record. Write the summary in your own words; nothing here is generated.

Summary of treatment and outcome

Demo text: course completed; stairs reported easier by the patient.

Home advice

Follow-up

Sign discharge summary

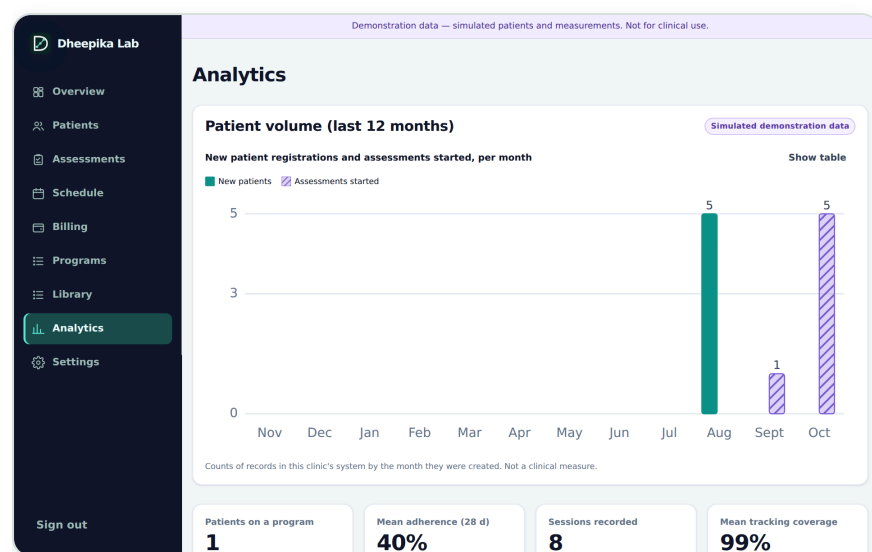
Download PDF

Discharge summary: physiotherapist's text above, the assembled summary below

- 1 Open the patient and press **Discharge**.
- 2 The summary fills itself from the record: visits attended, the treatment course, the patient's first and latest pain scores, the latest assessment's measurements, and the signed clinical impression.
- 3 Write the **summary of treatment and outcome**, **home advice** and **follow-up** in your own words. Nothing in these boxes is generated.
- 4 Press **Sign discharge summary**, then **Download PDF**. Signed versions are kept and cannot be edited; signing again adds a new version.

It goes back to "preliminary" when data changes. If a visit is marked attended, a capture is added or a course changes after signing, the summary shows "Signed version out of date" until you sign again.

Analytics and settings



Analytics

Analytics

- **Patient volume (last 12 months):** a bar chart, with a table view.
- **Mean change in camera-estimated ROM** (first → latest session).
- **Measurement quality** of the captures.
- **Attendance and follow-up (last 6 months):** attendance rate ($\text{attended} \div (\text{attended} + \text{missed})$), missed visits, active treatment courses, and courses with **no next booking**. A monthly chart shows attended, missed and cancelled visits. Past visits still marked "scheduled" are flagged so you can record them; they are not counted either way.

Settings

SECTION	USE
Clinic server	Sync status ("Up to date", changes waiting, last synced) and Sync now .
Clinic	Clinic name, the emergency number shown to patients, and whether payment records are kept when a patient deletes their account (off by default). Release readiness and incidents on this device are listed below it.
Observation thresholds	The thresholds that turn a camera estimate into a movement observation. Unreviewed rules are counted on the Overview.
Clinical rule sets & protocols	Versions and approval status. Draft rule sets are labelled "not clinically approved".
Motion engine (this device)	Body-tracking model: Lite (fastest), Full , or Heavy (most accurate; a ~31 MB download, best on a laptop).
Demonstration data	How demo patients are flagged. Sign in with a real account to remove demo data.
Data boundary · Audit log	What stays on the device and what is shared; the latest 40 audit entries.

Attendance and follow-up (last 6 months)

Open schedule

Attendance rate

75%

attended ÷ (attended + missed)

Missed visits

1

Active treatment courses

1

Active, no next booking

0

Visits per month by recorded outcome

Show table

■ Attended ■ Missed ■ Cancelled

Month	Attended	Missed	Cancelled
May 26	0	0	0
Jun 26	0	0	0
Jul 26	0	0	0
Aug 26	0	0	0
Sept 26	2	1	0
Oct 26	1	0	0

PATIENT	COURSE	ATTENDED / PLANNED	LAST VISIT	NEXT BOOKING
Demo patient DP-01 PT-4EBE04	Knee rehabilitation (demo)	3 / 8	2026-10-04	2026-10-08

Counted from appointment records only. A missing booking is an administrative prompt to contact the patient, not a clinical judgement.

Analytics: attendance and follow-up

Validation study

Pairs camera metrics with clinician reference measurements (entered under each capture). Participants are assigned to a **tuning** or **final evaluation** split, never both. Demo and simulated data are excluded. See docs/validation/STUDY_PROTOCOL.md.

Release thresholds (not set)

i

The clinical lead sets these before any final-evaluation data is analysed. They are not pre-filled: there is no evidence-based default to offer.

METRIC	LOA WITHIN ± (UNITS)	MAX FAILURE %	MIN ICC	MIN N
heel raise count				
heel raise height angle				
hip abduction peak				
hip abduction trunk lean				

Settings: validation study (thresholds start empty; the clinical lead sets and locks them)

Settings: validation study

For the accuracy study only. Release thresholds are entered and **locked by the clinical lead** before final-evaluation data exists. Results show agreement (bias, limits of agreement), capture failure with reasons, **test–retest** repeatability from a second session on a later day, missing data, and agreement by device, view, lighting, clothing, sex, age band and (only with consent) skin-tone band. Reference measurements, with these conditions, are entered under each capture in the assessment workspace.

The study figures describe the records entered. They are evidence only inside the locked study protocol (docs/validation/STUDY_PROTOCOL.md), analysed once, in the evaluation split.

Analytics: where capture fails

Counts of camera captures that failed the quality check and of values that were withheld, by protocol version, camera view and device type (Android, iOS, desktop), with the most common reasons. Use it to improve set-up, lighting and phone placement. Demo records are always left out, so the panel stays empty until real captures exist. **It counts failures only; it does not show how accurate the values that passed are.**

Where capture fails

✓ Last 90 days

All time

Counts of camera captures and values that were withheld, and the reasons. Use them to improve set-up, lighting and phone placement. They do not show how accurate the values that passed are. 36 demo records are excluded.

No real (non-demo) camera captures in this period yet. Controlled tests with volunteers can be recorded with the [occlusion challenge recorder](#).

Analytics: where capture fails (demo data excluded)

Settings: outcome questionnaires

A register of the questionnaires the clinic may want to use. It holds details only: name, version, licence position, licence holder, licence terms and the official scoring manual. **Start using** stays disabled until all of these and your clinical approval of that exact version are recorded. Changing the version or licence withdraws the approval and switches the questionnaire off. Questionnaire wording and scoring are added later as a separate, reviewed release from the licensed official materials, never typed in from memory.

Outcome questionnaires

Standard questionnaires are usually copyrighted. Their wording and scoring must come from the licensed official materials, not be typed in from memory. Record the licence position here. A questionnaire can be switched on only when the licence, the official scoring source and a clinical approval of this version are all recorded. Changing the version or licence withdraws the approval.

Questionnaire name

Version

Add to registry

▼ Example knee questionnaire v1.0 Not in use

Licence position

Not checked

Licence holder

Licence document, agreement number or terms URL

Official scoring manual (title, edition)

Clinical approval: not recorded

- Licence: not checked
- Licence holder not recorded
- Licence document or terms not recorded
- Official scoring source not recorded
- No clinical approval of this version

Record my clinical approval of v1.0

Start using

Questionnaire content and scoring are added as a separate, reviewed release once the official materials are in hand.

Settings: a questionnaire that cannot be used yet, with the reasons

How your data moves

ACCOUNT	CAN SEE	CAN CHANGE
Physiotherapist	Every patient of the clinic, on every device	Everything in the clinic, except history, which can never be edited
Patient	Only their own records	Only their own records. Never plans, impressions, reviewed reports or clinic settings
Demo	Simulated data only	On the device only. Never sent to the server
Schedule and billing	Physiotherapists. Clinic expenses are never visible to patients	Physiotherapists only

What is saved

- Answers, pain scores and check-ins.
- Body-point coordinates and calculated measurements.
- Posture stills, only with the optional image consent.
- Reviews, impressions, programs and an audit log of every access and change.

What is never saved

- Video. Frames are processed in memory and discarded.
- Demo data on the server.
- Your password in the app. Sign-in is handled by the clinic server.

Syncing

- Every change is queued and sent to the clinic server. Changes from other devices are fetched about every 20 seconds, when the connection returns, and when you reopen the app.
- **Offline?** A banner says how many changes are waiting. They are kept on the device and sent automatically later.
- If the same record is changed in two places at once, the later change is kept.

Leaving

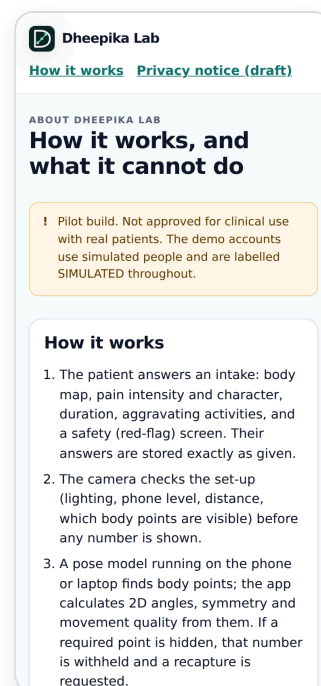
- **Sign out** removes this account's records from the device.
- **Profile → Delete my account and data** permanently removes the patient's records and sign-in from the server and the device. If the clinic has switched on **Keep payment records** (Settings → Clinic, off by default), payment amounts, dates, method and receipt reference are kept for accounting with the patient link, course name and notes removed.
- **Profile → Export my data** downloads a copy.

About this app and the privacy notice

- **How it works & accuracy** (/about, linked from the Welcome screen and Profile): how the app works, a data-flow diagram, accuracy and validation status (**not yet validated**), regulatory status (not assessed by any regulator), the guidance it refers to (not endorsements), device requirements and questions.
- **Privacy notice (draft)** (/privacy): written against India's Digital Personal Data Protection Act 2023 for legal review. It covers who is responsible, what is collected and why, each separate consent, how long data is kept, your rights, security and breaches. Items in [brackets] are for the clinic to complete. **It is not in force** until reviewed.

New kinds of data

WHAT	WHERE IT GOES
Desk posture check	Three camera estimates, saved only when the patient taps Save; sent for physiotherapist review. No image.
Walk-in registration	The details staff enter; no login is created.
Study conditions	Lighting and clothing with a reference measurement; skin-tone band only with the participant's separate consent.
Device check, audit and benchmark files	Timings and device details only; no patient data.



How it works & accuracy

Safety and limits

Not for patient care yet. Dheepika Lab is a pilot. Clinical approval is on hold. Use test accounts and consenting volunteers only, until clinical approval, patient consent, a data-processing agreement and validation on real phones are in place.

The camera can

- Estimate 2D body points, joint angles, left/right symmetry and movement quality.
- Do this only when the needed joints are visible and the set-up checks pass.
- Count repetitions and compare with the target range.

The camera cannot

- Identify the tissue or condition causing pain.
- See bones or vertebrae. That needs X-ray, CT, MRI or ultrasound, read by qualified clinicians.
- Measure real distances (centimetres) from one camera.
- Give disease probabilities, severity grades or diagnoses.

Rules the app always follows

- 1 No number without a good capture.** If capture fails, the value is withheld, the reason is shown and a recapture is requested.
- 2 Sources are never mixed.** Patient reports, camera estimates, algorithmic observations and clinician findings are always labelled and kept apart.
- 3 Every observation shows its working:** measurement, view, confidence, calculation, version and limitations, in the "Why?" view.
- 4 The physiotherapist decides.** Drafts and observations are proposals until a clinician reviews them. Rule sets marked DRAFT still need clinical-lead approval.
- 5 Demo is always labelled.** Simulated data shows a purple "Demonstration data" banner and SIMULATED tags.

For patients

- Exercise in a clear space, with a chair or wall nearby for balance.
- Stop if pain is sharp or getting worse, or if you feel dizzy or unwell.
- The check-in is not monitored in real time. For urgent problems, call your clinic or emergency services.

Troubleshooting

It keeps saying "move closer" or "step back"

Read the Distance line in the checklist: it shows the percentage of the picture your body fills and the range to aim for. Stand so the whole body, head to feet, is in the picture, at the distance the test states (usually 2–3 m). Keep the phone upright. If it still happens, take a screenshot of the checklist and send it to the clinic.

The camera does not start

Allow camera access when the browser asks. If you refused before, allow it in the browser's site settings. Check you gave camera consent in Profile → Privacy & consent. Close other apps that use the camera.

A value shows "—"

A joint the value needs was hidden, out of the picture or unsteady. See "Why a value shows —" in section 08, fix the set-up and recapture.

"Tests are paused"

Your safety answers need your physiotherapist's review first. They will contact you.

The first scan is slow

The body-tracking model downloads once. The Heavy model (Settings) is ~31 MB; use Lite or Full on phones.

Desk check shows "Withheld"

Turn fully side-on so your ear, shoulder and hip on the camera side are visible; for the hip angle, the knee must be visible too.

The email link says it is invalid or expired

Links work once and expire. Sign in normally, or use Forgot password to get a new link.

I signed up but I don't see the physiotherapist screens

Your email must be on the clinic's physiotherapist list. Ask the clinic owner to add it, then sign out and in again.

"Changes are not yet saved to the clinic server"

You are offline or the server was unreachable. Your changes are kept on the device and sent automatically. A physiotherapist can press Settings → Clinic server → Sync now.

A patient's data is missing on the physio laptop

Check both devices are signed in to real accounts, not demo. Demo data never leaves the device. Wait about 20 seconds or press Sync now.

The report went back to "preliminary"

A capture or review changed after approval. Check the changes and approve again.

Text is too small

Profile → Large text, or use the browser's zoom.

The 3D body switched to the 2D map

The device could not start 3D graphics or ran short of graphics memory. Your choices are kept. Use the 2D map, or tap 3D body to retry on a stronger device.

The 3D body is slow to appear

The light model is about 6 MB and the full one about 20 MB, downloaded the first time. On mobile data, use the light model or the 2D map. To use 3D without internet: Anatomy → **Offline use** → **Keep light model**.

A banner says a new version is ready

Finish what you are recording, then tap **Reload now**. The app never updates itself mid-capture.

For testers: checking on real phones

These tools help the clinic and testers collect the real-phone evidence the app still needs. None of them is a pass/fail judgement; criteria are set by the product owner and clinical lead before testing.

TOOL	HOW	RECORD IN
3D atlas device check	Anatomy → Device check. Open the 3D body, turn it through all views, tap ten areas, use it for 5 minutes, then Download device record .	docs/ANATOMY_PHONE_QA.md
Camera benchmark	/lab.html → run on the phone's mount. Each result now records start/end time, browser, device, battery start/end (where the browser allows) and whether the page stayed open.	docs/tracking/PHONE_RUN.md
Accessibility audit	Developers: npm run build , npm run serve , then npm run a11y . It checks 23 screens at phone and desktop width against WCAG 2 A/AA rules.	Report file a11y-report.json
Validation study	Settings → Validation study, with reference measurements under each capture.	docs/validation/STUDY_PROTOCOL.md
Occlusion challenge recorder	Analytics → Where capture fails → occlusion challenge recorder . Volunteers only (next page).	Phase 22 challenge set
Automated checks	Every code change runs unit tests, browser journeys on a phone and a desktop profile, the accessibility audit, licence and security checks, and a size report.	GitHub Actions; docs/PHASES_41_60.md

Automated checks are not user testing. Sessions with TalkBack and VoiceOver users on real phones, a qualified Tamil clinical review, and validation with consenting volunteers on the clinic's phones are still required before any accuracy or accessibility claim.

Occlusion challenge recorder

New clip

Participant code (no names)

☒ The volunteer has given written consent to landmark-only recording for occlusion testing (consent form challenge-consent-draft-1, draft awaiting ethics review). They are not a patient of this clinic in this recording.

What covers the joint

Joints that will be covered

Camera view

Lighting (optional)

Notes (optional, no identifying details)

Open camera

Recorder set-up

For testing on **volunteers only, never patients**, with their written consent. The recorder shows what happens when a phone, hand, chair or bag covers a joint. It keeps only the body-tracking points, never video or pictures.

- 1 Enter a participant code (no names) and confirm consent.
- 2 Choose what covers the joint, which joints, and the camera view.
- 3 **Open camera** → **Start recording**. Begin with the joints clear, then cover them. **Hold** the big button (or the space bar) for exactly the time the cover is in place.
- 4 Review the clip's checks, then **Save clip**.
- 5 **Export (JSON)** for the Phase 22 reviewers. **Export and lock** makes the clips unchangeable; do this only when the reviewers have agreed the set.

Clips recorded with the simulated camera are marked "simulated" and can never be exported. The consent form is a draft that needs ethics review before use.

Quick reference cards

Patient: 10 steps

1. Open the site → **Get started** → confirm your email.
2. Setup: About you → consent → your concern → choose an area.
3. **Symptom map**: tap areas, choose symptoms, draw spreading pain.
4. **History**: pain now and worst in 24 h, feel, duration, triggers.
5. **Safety check**: answer honestly.
6. Phone upright on a stand where the test says (usually 2–3 m), whole body in view, light in front.
7. **Movement tests**: wait for all green ticks; recapture any **X**.
8. **Submit** to your physiotherapist.
9. When your program appears: Home → **Start session**; check in daily.
10. Stop if pain is sharp or worsening. Emergency → call for help.

Physiotherapist: 12 steps

1. Get your email added to the clinic list → sign up or sign in.
2. **Overview**: clear critical alerts and the exception queue first.
3. **Assessments**: open the oldest submitted assessment.
4. History & safety → check flags and the original answers.
5. Captures & replay → Accept / Reject / Repeat each estimate.
6. Left / right or baseline → current.
7. AI draft → Accept / Edit / Reject / Defer each statement.
8. Evidence & reasoning → examine, use "Why?", sign the impression.
9. Report → Approve as clinician-reviewed → download PDF.
10. **Programs** → build, set pain-pause and reassessment → publish.
11. **Schedule** → book or walk-in, mark attended; **Billing** → record payments.
12. End of care → patient → **Discharge** → write, sign, download.

Glossary

TERM	MEANING
Body points (landmarks)	33 points the camera model finds, such as shoulders, hips, knees and ankles.
ROM	Range of motion: how far a joint moves, in degrees.
Camera estimate	A value calculated from body points. It is not a clinical measurement until reviewed.
Algorithmic observation	A camera estimate that crossed a clinic-set threshold. A prompt for review, not a finding.
Valid capture	A recording that passed the quality checks (joints visible, steady, enough frames).
Plumb line	The vertical reference line from the ankles on the posture scan.
FPPA	Frontal plane projection angle: knee alignment seen from the front during a squat.
Clinical impression	The physiotherapist's signed conclusion. Only this appears as a conclusion in the report.
Treatment course	The number of sessions and the fee agreed with a patient; attended sessions are counted from the schedule.
PT code	The patient's short ID in the directory, for example PT-3F9A21.
Test–retest repeatability	How closely two sessions on different days agree for the same person and set-up (ICC, SEM, MDC95).
Walk-in	A visit recorded at the time it happens, already marked attended.
Device record	A file of timings from the 3D atlas on one device, used for phone testing. No patient data.

Dheepika Lab · physiovision-ai-eta.vercel.app · Guide version 3.1 · 7 October 2026. Screens show simulated demonstration data. This guide describes a pilot; it is not clinical advice.